

**Akash Institute of Medical Sciences &
Research Centre (AIMSRC)**

Patient Name:	MAMATHA^34Y/F	Age/Gender:	N/A/N/A	Patient ID:	3215586
Study Date:	N/A	Ref. Doctor:	john	Accession No:	65800
Reported Date:	16/2/2026, 1:31:05 pm	Modality:	CR	Body Part:	CHEST

**CR CHEST
REPORT**

History:

Patient presented with chest pain or trauma. Routine chest examination requested. Patient presented for routine chest radiograph evaluation. No acute respiratory complaints at the time of examination.

Findings:

The trachea is central in position. Both lung fields are well expanded and show normal radiolucency. No focal consolidation, mass lesion, or nodular opacity is identified in either lung. The bronchovascular markings are within normal limits. There is no evidence of collapse or segmental atelectasis.

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Both costophrenic angles are clear and sharp. No pleural effusion is detected. There is no pneumothorax. The hemidiaphragms are smooth and well defined. No subdiaphragmatic free air is seen.

The bony thoracic cage, including ribs, clavicles, scapulae, and visible spine, shows no obvious fracture or lytic/sclerotic lesion. Soft tissues of the chest wall appear unremarkable. No mediastinal widening is noted. Overall lung parenchyma appears normal without any interstitial infiltrates.

Key Images:



Conclusion:

Normal chest radiograph. No acute pathology detected.

No radiographic evidence of active pulmonary disease is seen. The lung fields appear clear bilaterally. Cardiac size is within normal limits. No pleural effusion or pneumothorax is identified.

No focal consolidation or infective changes are detected. No features suggestive of tuberculosis are noted. There is no mass lesion or suspicious pulmonary nodule. Hilar structures are unremarkable.

The mediastinum appears normal without widening. Diaphragmatic contours are normal. Costophrenic angles are clear. Bony thoracic cage appears intact.

Overall, the chest radiograph is within normal limits. No acute cardiopulmonary abnormality is detected on this study.

Reported By:

ANJALI

anjali

MBBS

Signed on 16/2/2026, 12:25:35 pm

Approved By:

Lekhana

Lekhana M N

MBBS

Signed on 16/2/2026, 12:55:13 pm

